

Hospital Price Transparency Analysis

Evaluation of Reporting Completeness, Code Standardization, and Cross-Hospital Pricing Comparisons

Jake Sheeler

Hospital Price Files Finder
(Patient Rights Advocate)

718,778 Records | 5 Hospitals

Duke Raleigh Hospital

WakeMed

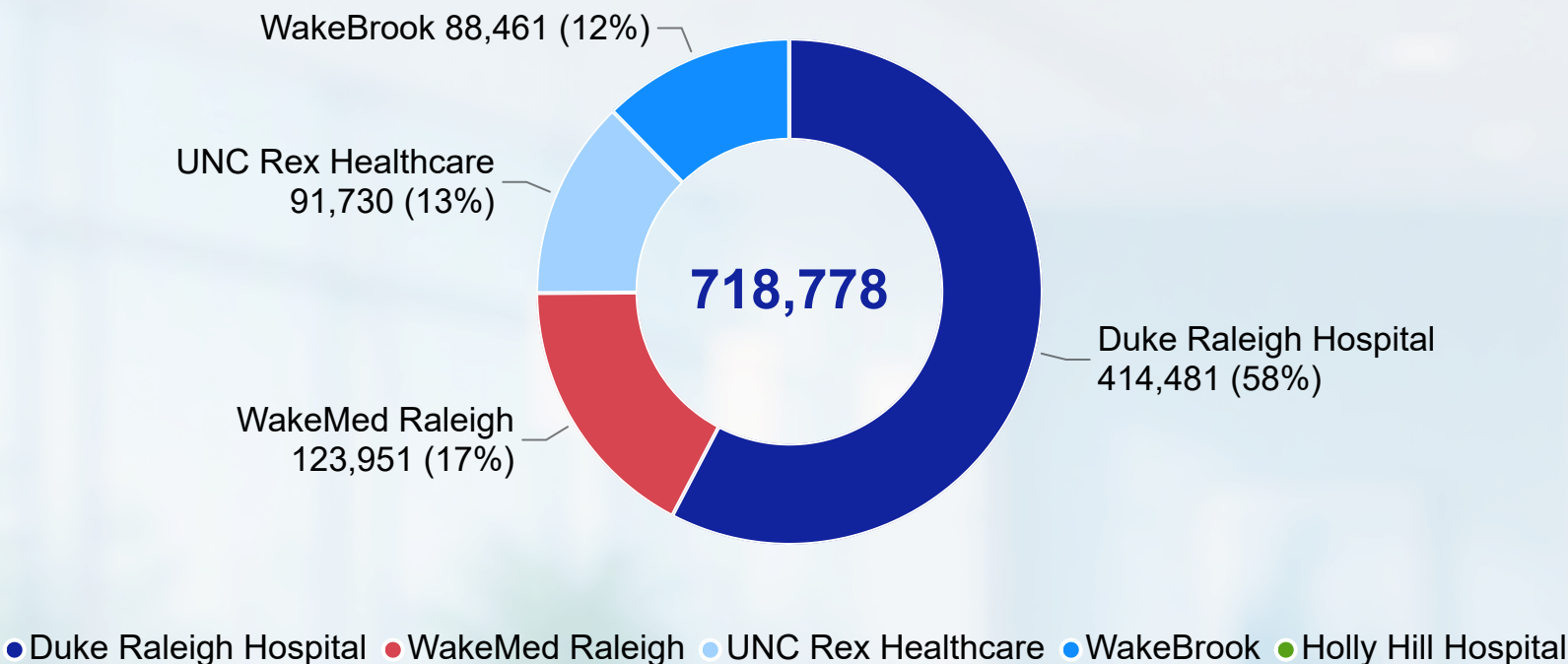
UNC REX
UNC HEALTH CARE

WakeMed
Mental Health & Well-Being
Hospital - WakeBrook

Holly Hill
HOSPITAL

Published Pricing Records by Hospital

Distribution of 718,778 published pricing records across five Raleigh-area hospitals



640

Comparable MS DRG
Codes

Shared Across 3+
Hospitals



Objective

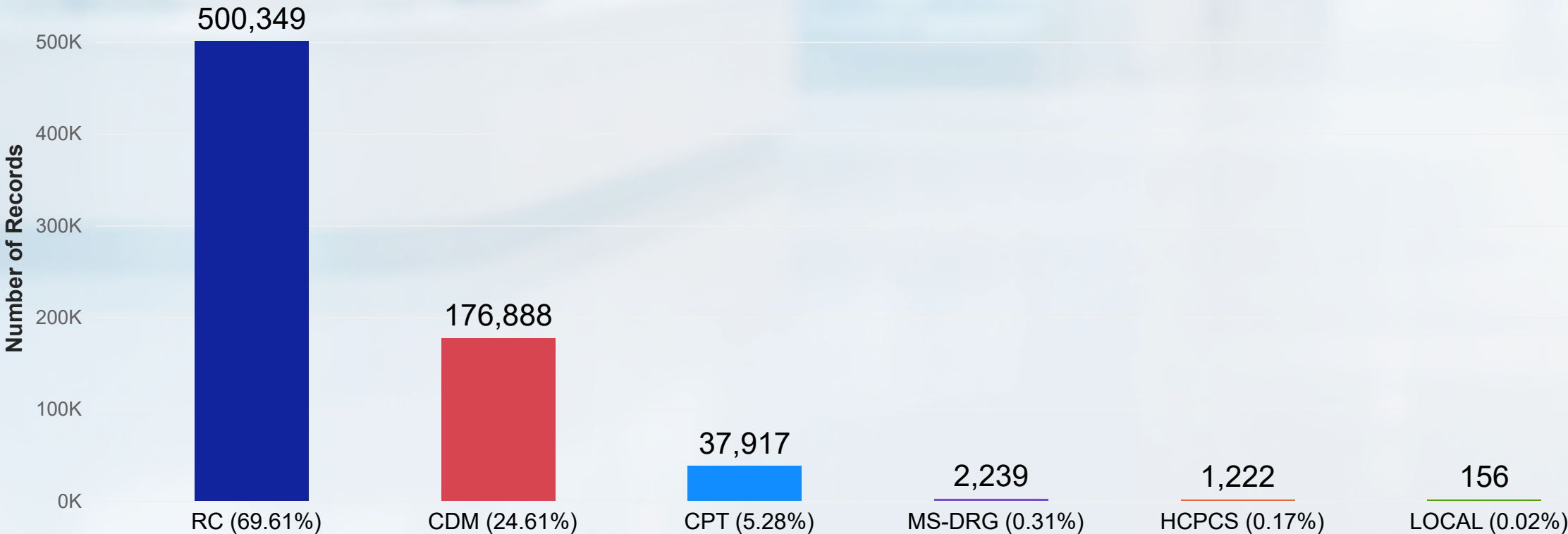
Evaluate hospital pricing transparency by measuring reporting completeness, standardizing procedure codes, and identifying comparable services across hospitals for pricing analysis.



Key Findings

- The dataset contains 718,778 published pricing records from five Raleigh-area hospitals.
- Duke Raleigh Hospital contributes 57.66% of all records in the dataset.
- Revenue Codes (RC) and Chargemaster Codes (CDM) account for more than 94% of all published records.
- 640 MS DRG procedures were identified across at least three hospitals, creating a reliable comparison group for cross-hospital price analysis.
- Holly Hill Hospital contributed 155 records (0.02% of the dataset).

Records by Procedure Code Type



Reporting Completeness and Data Availability

Comparison of published gross, cash, minimum, and maximum charge coverage by hospital

Hospital Name

All



3

Hospitals With Complete
Primary Price Coverage



35,507

WakeMed Raleigh Records
Missing Gross and Cash Charges



17

WakeBrook Records
With Negotiated Ranges

Published Pricing Field Coverage by Hospital

Percentage of records containing published price information

Hospital Name	Gross Charge	Cash Charge	Minimum Charge	Maximum Charge
Duke Raleigh Hospital	100.00%	100.00%	100.00%	100.00%
Holly Hill Hospital	100.00%	100.00%	100.00%	100.00%
UNC Rex Healthcare	100.00%	100.00%	100.00%	100.00%
WakeMed Mental Health & Well-Being Hospital	99.98%	99.98%	0.02%	0.02%
WakeMed Raleigh Campus	71.35%	71.35%	100.00%	100.00%

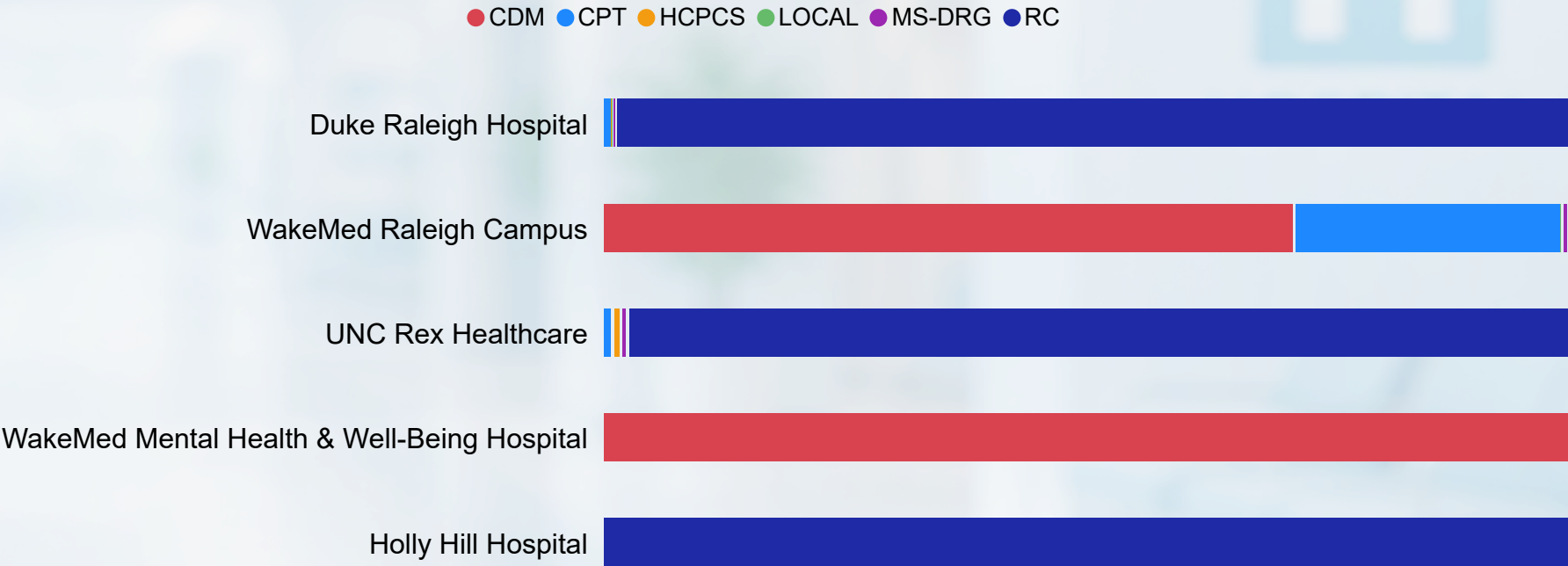
95% - 100% (Complete)

50% - 94.99% (Partial)

Below 50% (Low)

Procedure Code Composition by Hospital

Composition of published pricing records by procedure code type



Key Findings

Recap

- Duke Raleigh Hospital, UNC Rex Healthcare, and Holly Hill Hospital reported complete coverage across all four primary pricing fields.
- WakeMed Raleigh Campus reported complete negotiated minimum and maximum charges, but gross and cash prices were unavailable for 35,507 records.
- WakeMed Mental Health & Well-Being Hospital reported gross and cash prices for nearly all records, but negotiated minimum and maximum charges were available for only 17 records.
- Procedure code composition varied significantly across hospitals, reducing direct comparability without standardization.
- Standardized MS DRG procedures provide the most reliable foundation for cross-hospital pricing analysis.

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Comparable MS DRG Pricing Analysis

Comparison of 636 Matching MS DRG procedures reported by Duke Raleigh Hospital and UNC Rex Healthcare

640

Comparable MS DRG Codes

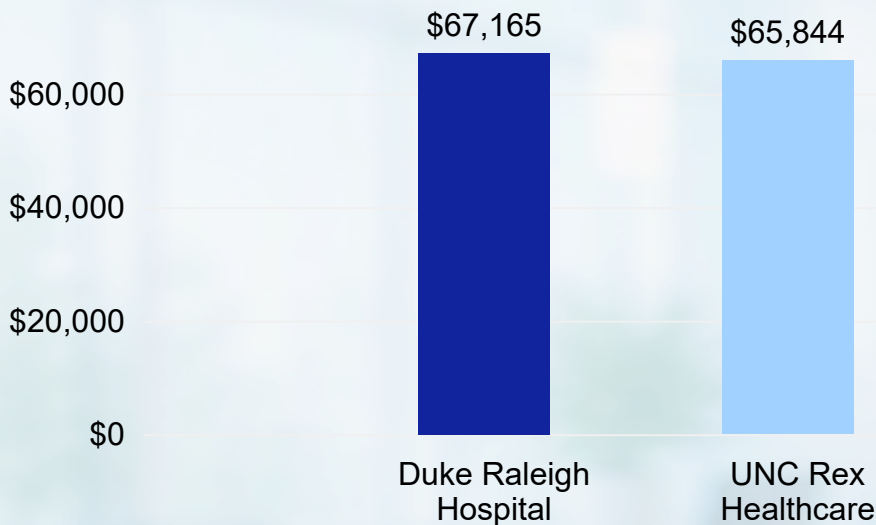
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DRGs Shared by 4 Hospitals

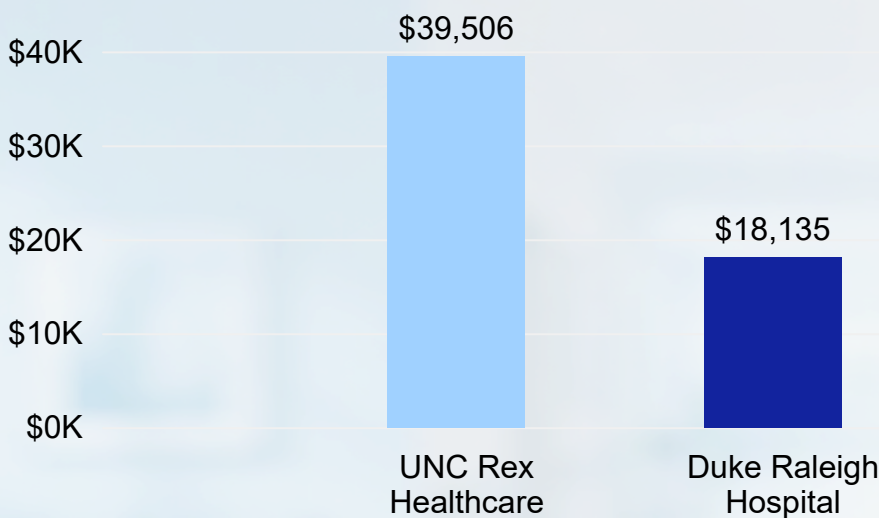
627

DRGs Shared by 3 Hospitals

Average Gross Charge by Hospital

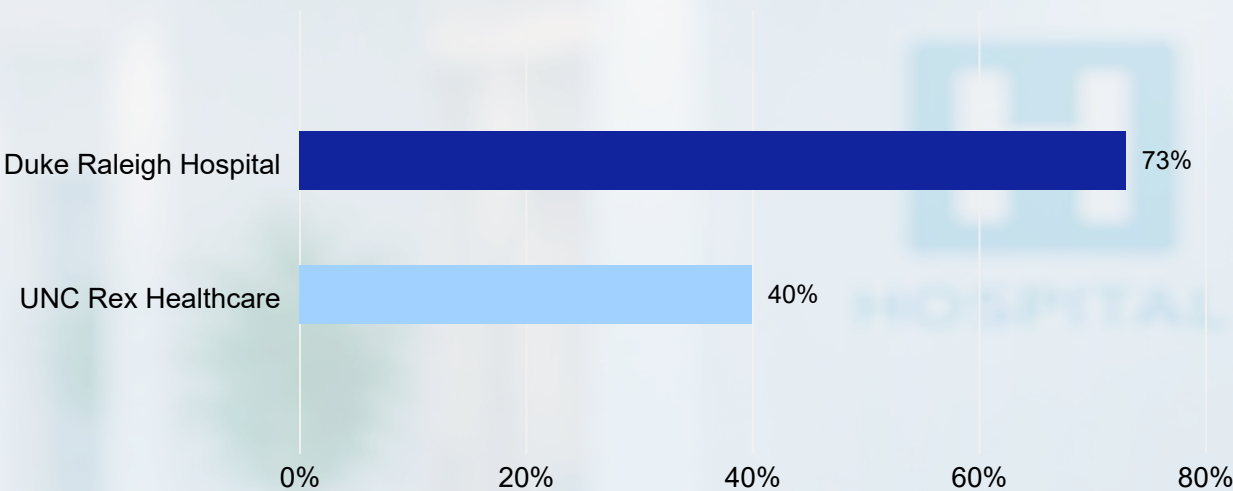


Average Cash Charge by Hospital



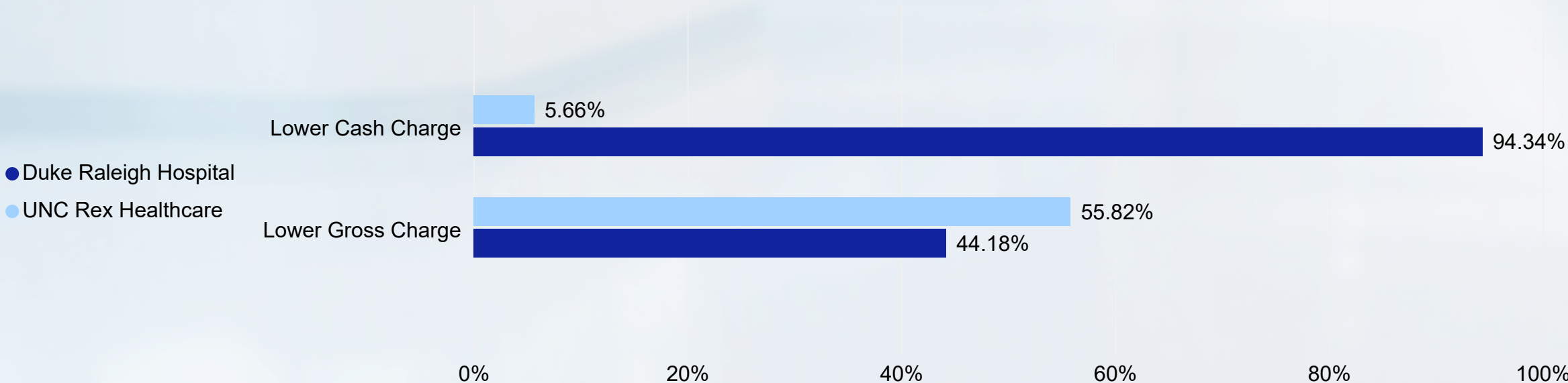
Average Cash Discount by Hospital

Average published self-pay discount across 636 matching MS DRG procedures



Lower Price Comparison

Percentage of 636 matching MS DRG procedures



Key Findings

- UNC Rex Healthcare reported the lower gross charge for 55.82% of matching MS DRG procedures.
- Duke Raleigh Hospital reported the lower cash price for 94.34% of matching procedures.
- Duke Raleigh and UNC Rex Healthcare had similar average gross charges.
- Duke Raleigh applied a substantially larger average cash discount, resulting in significantly lower published cash prices.
- The average published cash charge at Duke Raleigh Hospital was approximately \$21,372 lower.

MS DRG Procedure Price Explorer

Interactive comparison of published hospital charges

Hospital Name

All

DRG Code

871

Procedure Description

All

Displayed Selection

DRG Code: 871

Procedure: Septicemia Or Severe Sepsis Without Mv >96 Hours With McC

Hospital: All Hospitals



MS DRG Gross Charge Difference Analysis

Analysis of matching MS DRG procedures with the largest published gross charge differences between Duke Raleigh Hospital and UNC Rex Healthcare.

Top 20 Largest Gross Charge Differences

DRG Code	Procedure Description	Duke Gross Charge	UNC Gross Charge	Gross Charge Difference	Higher Priced Hospital
820	Lymphoma And Leukemia With Major O.R. Procedures With McC	\$516,478.06	\$84,433.60	\$432,044.46	Duke Raleigh Hospital
799	Splenic Procedures With McC	\$131,864.37	\$438,033.91	\$306,169.54	UNC Rex Healthcare
790	Extreme Immaturity Or Respiratory Distress Syndrome, Neonate	\$421,606.92	\$123,328.45	\$298,278.47	Duke Raleigh Hospital
1	Heart Transplant Or Implant Of Heart Assist System With McC	\$1,122,336.11	\$860,995.07	\$261,341.04	Duke Raleigh Hospital
97	Non-Bacterial Infection Of Nervous System Except Viral Meningitis With McC	\$101,336.47	\$354,953.90	\$253,617.43	UNC Rex Healthcare
40	Peripheral, Cranial Nerve And Other Nervous System Procedures With McC	\$108,105.41	\$352,261.95	\$244,156.54	UNC Rex Healthcare
20	Intracranial Vascular Procedures With Principal Diagnosis Hemorrhage With McC	\$291,191.38	\$527,866.69	\$236,675.31	UNC Rex Healthcare
3	Ecmo Or Tracheostomy With Mv >96 Hours Or Principal Diagnosis Except Face, Mouth And Neck With Major O.R. Procedures	\$713,587.20	\$480,163.55	\$233,423.65	Duke Raleigh Hospital
61	Ischemic Stroke, Precerebral Occlusion Or Transient Ischemia With Thrombolytic Agent With McC	\$97,969.31	\$324,403.23	\$226,433.92	UNC Rex Healthcare
278	ULTRASOUND ACCELERATED AND OTHER THROMBOLYSIS OF PERIPHERAL VASCULAR STRUCTURES WITH MCC	\$36,773.16	\$259,109.46	\$222,336.30	UNC Rex Healthcare
834	Acute Leukemia With McC	\$252,445.54	\$38,525.35	\$213,920.19	Duke Raleigh Hospital
548	Septic Arthritis With McC	\$38,947.31	\$240,064.65	\$201,117.34	UNC Rex Healthcare
969	Hiv With Extensive O.R. Procedures With McC	\$266,322.79	\$95,303.85	\$171,018.94	Duke Raleigh Hospital
317	CONCOMITANT LEFT ATRIAL APPENDAGE CLOSURE AND CARDIAC ABLATION	\$44,174.43	\$214,835.96	\$170,661.53	UNC Rex Healthcare
876	O.R. Procedures With Principal Diagnosis Of Mental Illness	\$192,121.80	\$23,234.94	\$168,886.86	Duke Raleigh Hospital
74	Cranial And Peripheral Nerve Disorders Without McC	\$31,250.77	\$196,605.85	\$165,355.08	UNC Rex Healthcare
307	Cardiac Congenital And Valvular Disorders Without McC	\$16,738.56	\$181,049.05	\$164,310.49	UNC Rex Healthcare
276	Cardiac Defibrillator Implant With McC Or Carotid Sinus Neurostimulator	\$332,325.46	\$168,537.34	\$163,788.12	Duke Raleigh Hospital
266	Endovascular Cardiac Valve Replacement And Supplement Procedures With McC	\$304,784.97	\$144,044.95	\$160,740.02	Duke Raleigh Hospital
835	Acute Leukemia With Cc	\$200,378.88	\$48,536.40	\$151,842.48	Duke Raleigh Hospital

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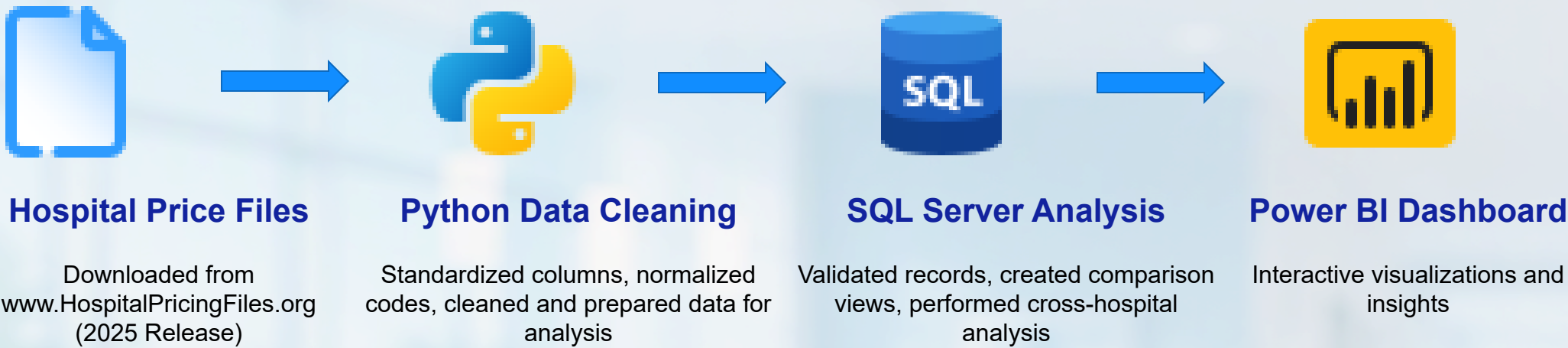
Holly Hill
HOSPITAL

Methodology & Limitations

Project workflow, data preparation process, definitions, and important analytical considerations.

Project Workflow

From raw hospital files to actionable insights



Key Definitions

Gross Charge

Published full hospital charge before discounts.

Cash Charge

Published self-pay rate available to uninsured patients.

Minimum Charge

Lowest negotiated payer rate reported.

Maximum Charge

Highest negotiated payer rate reported.

MS DRG

Medicare Severity Diagnosis Related Group used to standardize inpatient procedures.

RC Code

Revenue Code used to classify hospital services and supplies.

Comparable Procedure

A procedure that could be matched across multiple hospitals using a standardized MS DRG code.



Key Limitations

Important considerations when interpreting the data

- Published prices do not reflect what a patient will actually pay, as final costs vary based on insurance coverage, negotiated rates, and individual circumstances.
- Hospitals reported pricing information using different coding structures and reporting approaches.
- Not all hospitals reported every pricing field for each procedure.
- Holly Hill Hospital reported Revenue Codes only and could not participate in MS DRG comparisons.
- WakeMed hospitals reported limited gross and cash charge information for many comparable MS DRG procedures.

Project Summary

This project examined whether hospital pricing files provide the level of transparency needed for meaningful healthcare price comparisons. The analysis identified major differences in reporting completeness, coding structures, available pricing fields, and published charges across five Raleigh area hospitals.

While patients are the intended beneficiaries of price transparency, the findings are most immediately useful to healthcare organizations, researchers, and regulators. These groups have the ability to improve reporting practices, establish clearer standards, and hold hospitals accountable when required pricing information is incomplete or difficult to use.

Additional reporting requirements, standardized file submissions, consistent procedure identifiers, and stronger enforcement are needed to improve the quality and comparability of hospital pricing data. Public access alone is not enough if the information cannot be understood or compared confidently.